

Public Health Emergency Operations Center

"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°078/MVE B_31/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (49)	- Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba - Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. - South Kivu (1/34): Miti-Murhesa - Tshopo (4/23): Kabondo, Lubunga, Makiso-Kisangani, Mangobo
Reporting date	July 31, 2026
Publication date	August 1, 2026

INDICATORS KEYS — CURRENT TO JULY 31, 2026

CONFIRMED CASES — 5 PROVINCES 3,674 +69 new cases (Ituri 53 · N-Kivu 12 · Haut-Uélé 4)	CONFIRMED DEATHS · LEATHITIS 1,621 (44.1%) +34 deaths (Ituri 25 · North Kivu 9)	SUSPECTED CASES OF THE DAY 321 Ituri 206 · N-Kivu 87 · Haut-Uélé 22 · South Kivu 6
PATIENTS IN ISOLATION 760 Ituri 525 · N-Kivu 196 · Haut-Uélé 37 · Tshopo 2	CURED — CUMULATIVE 666 +15 recovered (Ituri 13 · North Kivu 2)	NATIONAL CONTACT TRACKING 78.0% 14,436 / 18,508 views · Ituri 80.0% · N-Kivu 74.7% · H-Uélé 66.7% · South Kivu 100%

1. Epidemiological Highlights

- **Epidemiological situation:** No new health zones affected in the last 24 hours. The total remains at 49 affected health zones out of 140 across the five provinces. Tshopo province reports a fifth affected zone, Wanie-Rukula, where the 7th provincial case was confirmed on July 29; this classification is not included in the cumulative totals pending harmonization with the provincial health division.
- **Case trends:** In the last 24 hours, 69 new confirmed cases, 34 confirmed deaths, and 15 recoveries were recorded.
 - o **Ituri:** 53 new confirmed cases were notified in Bunia (16), Nizi (10), Fataki (8), Rwampara (6), Nia-Nia (6), Mongbwalu (4), Lita (1), Mangala (1) and Nyankunde (1), as well as 25 deaths, including 15 community deaths (60.0%). Eleven (11) positive retests were also returned.
 - o **North Kivu:** 12 new confirmed cases were reported in Beni (7), Katwa (2), Butembo (1), Kalunguta (1), and Musienene (1), as well as 9 deaths, including 5 community deaths. The provincial total reaches 378 confirmed cases and 251 deaths, representing a case fatality rate of 66.4%.

o **Haut-Uélé** : 4 new confirmed cases have been reported (Isiro 3 and Pawa 1) and no deaths were confirmed today. The provincial total now stands at 57 confirmed cases and 28 deaths, representing a case fatality rate of 49.1%, spread across 5 health zones out of 13.

o **Tshopo** : no new confirmed cases or deaths have been reported. The provincial total remains at 7 confirmed cases.

5 deaths and 1 recovery, representing a case fatality rate of 71.4%.

South Kivu : The provincial total remains at 3 confirmed cases, including 1 death and 2 recoveries. Contact tracing has resumed: 78 contacts listed, 23 discharged, and 55 still being monitored, all seen (100%), including 41 in quarantine; the note states, "These are contacts of a confirmed death imported from Tshopo province." The number of days without a new confirmed case in Miti-Murhesa is 52 days since June 10th, while the actual interval is 51 days.

- **Analysis**: Ituri remains the epicenter of the epidemic, accounting for 87.9% of cumulative cases and 76.8% of new daily confirmations. Contact tracing has risen to 78.0%, compared to 75.3% the previous day, but remains well below the operational threshold of 95%, while the isolation ward burden has increased to 760 patients, representing an overall occupancy rate of 72.4%.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 49 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

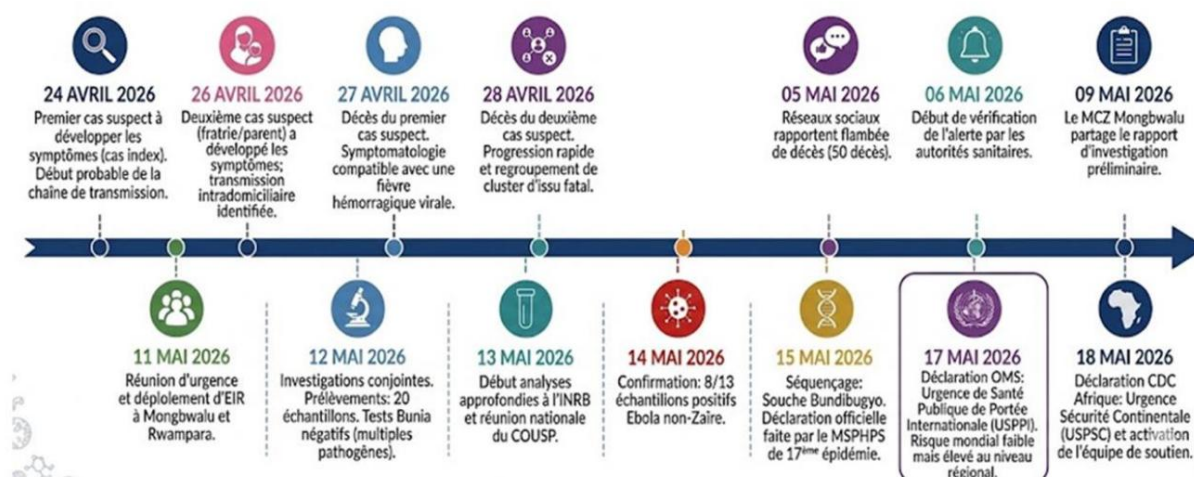


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	3,229	1,336	41.4%	28 / 36 (77.8%)	53
North Kivu	378	251	66.4%	11/34 (32.4%)	12
Haut-Uélé	57	28	49.1%	5/13 (38.5%)	4
Tshopo	7	5	71.4%	4/23 (17.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,674	1,621	44.1%	49 out of 140	69

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 87.9% of confirmed cases (n = 3,229) and 82.4% of deaths (n = 1,336), resulting in a case fatality rate of 41.4%. The health zones of Bunia (896 cases), Rwampara (633), Mongbwalu (545), Nizi (387), Lita (132), and Nyankunde (115) remain the most affected, accounting for 83.9% of reported cases at the provincial level and 73.7% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3,229	1,336	41.4%	53	15	10	25
Adja	11	1	9.1%	0	0	0	0
Aru	5	5	100.0%	0	0	0	0
Ariwara	7	2	28.6%	0	0	0	0
Aungba	9	5	55.6%	0	0	0	0
Bamboo	60	12	20.0%	0	0	0	0
Boga	1	1	100.0%	0	0	0	0
Bunia	896	259	28.9%	16	2	0	2
Damascus	17	8	47.1%	0	0	0	0
Drodro	10	6	60.0%	0	0	0	0
Fataki	35	17	48.6%	8	5	0	5
Gethy	1	0	0.0%	0	0	0	0
Kambala	2	0	0.0%	0	0	0	0
Kilo	29	11	37.9%	0	0	0	0
Komanda	36	25	69.4%	0	0	0	0
Lita	132	79	59.8%	1	0	4	4
Logo	9	3	33.3%	0	0	0	0
Lolwa	7	2	28.6%	0	0	0	0
Mahagi	4	1	25.0%	0	0	0	0
Mambasa	9	6	66.7%	0	0	0	0
Mandima	16	12	75.0%	0	0	0	0
Mangala	102	64	62.7%	1	0	0	0
Mongbwalu	545	259	47.5%	4	0	1	1
Nia-Nia	101	53	52.5%	6	1	2	3
Nizi	387	187	48.3%	10	7	1	8
Nyankunde	115	32	27.8%	1	0	1	1
Rimba	9	4	44.4%	0	0	0	0
Rwampara	633	258	40.8%	6	0	1	1
Tchomia	41	24	58.5%	0	0	0	0

North Kivu	378	251	66.4%	12	5	4	9
Blessed	57	46	80.7%	7	4	1	5
Butembo	83	65	78.3%	1	0	1	1
Goma	1	0	0.0%	0	0	0	0
Kalunguta	6	3	50.0%	1	0	0	0
Katwa	168	102	60.7%	2	0	2	2
Kyondo	13	8	61.5%	0	0	0	0
Mabalako	1	0	0.0%	0	0	0	0
Masereka	4	0	0.0%	0	0	0	0
Musienene	40	23	57.5%	1	1	0	1
Oicha	4	3	75.0%	0	0	0	0
Vuhovi	1	1	100.0%	0	0	0	0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%	0	0	0	0
Haut-Uélé	57	28	49.1%	4	0	0	0
Boma	12	4	33.3%	0	0	0	0
Mangbetu	19	9	47.4%	3	0	0	0
Isiro	10	7	70.0%	1	0	0	0
Pawa	1	0	0.0%	0	0	0	0
Rungu	15	8	53.3%	0	0	0	0
Tshopo	7	5	71.4%	0	0	0	0
Kabondo	1	1	100.0%	0	0	0	0
Lubunga	1	0	0.0%	0	0	0	0
Makiso-Kisangani	4	3	75.0%	0	0	0	0
Mangobo	1	1	100.0%	0	0	0	0
Total	3,674	1,621	44.1%	69	20	14	34

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Lita and Nyankunde, with transmission continuing in the areas already affected without any new geographical expansion in the last 24 hours.

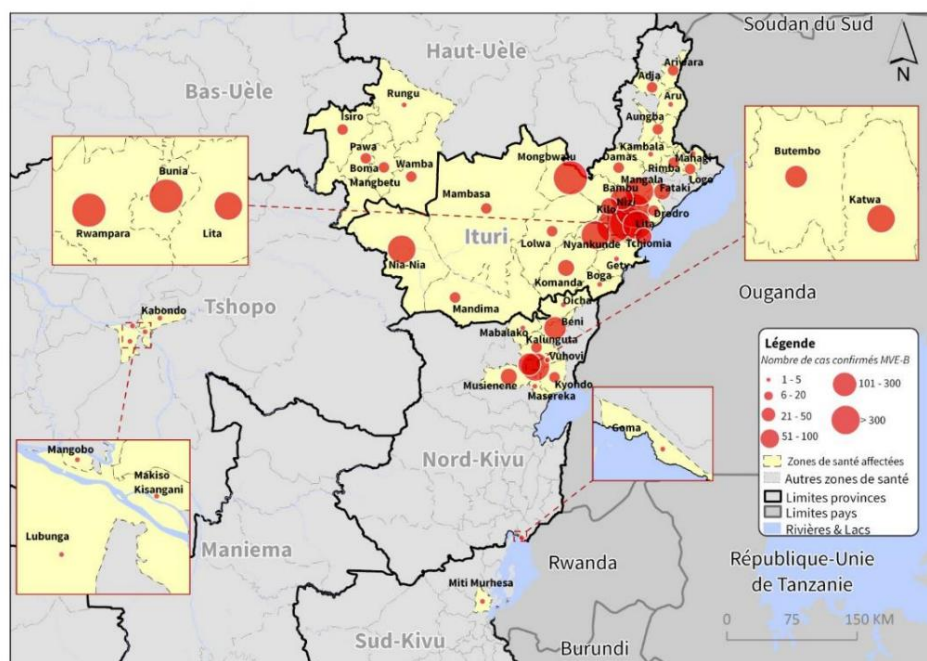


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

Transmission in already affected health zones underscores the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S26 to S31, with fluctuations related to notification delays and the gradual consolidation of data.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

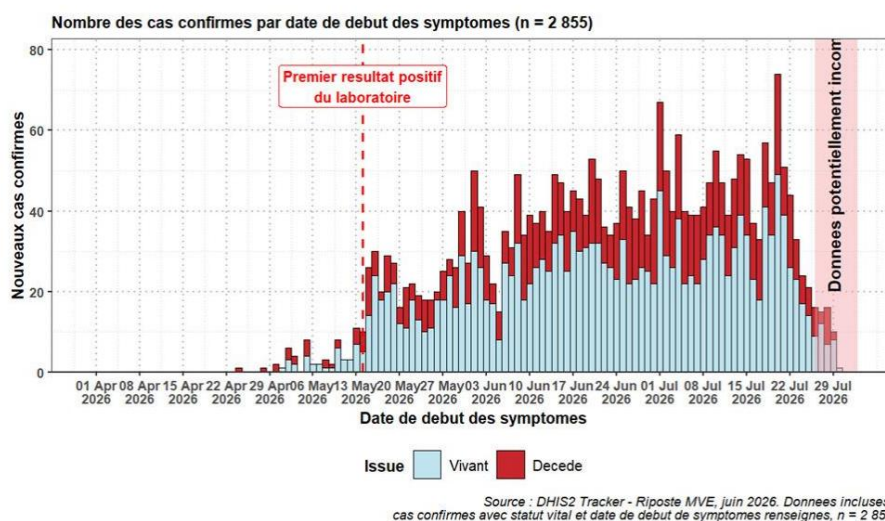


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

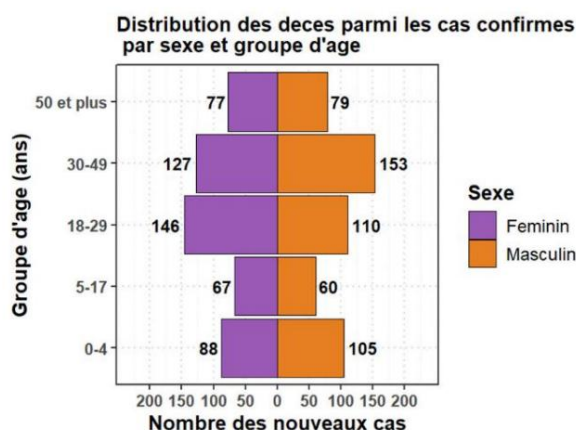
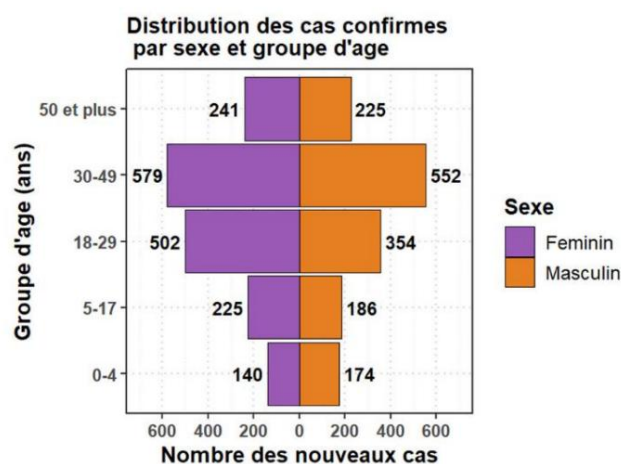


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	1	169	19	ND	49	238
New alerts received	530	864	3	ND	6	1,403
Total alerts for the day	531	1033	22	ND	55	1,641
Alerts investigated	408	852	22	ND	10	1,292
Investigation rate (24 h)	76.8%	82.5%	100.0%	ND	18.2%	78.7%
Alerts confirmed — live	125	38	21	ND	6	190
Alerts confirmed — deceased (comm.)	81	49	1	ND	0	131
Total suspected cases for today	206	87	22	ND	6	321

In total, 1,641 alerts were managed (including 1,403 new ones), 1,292 were investigated within 24 hours (78.7%), and 321 were validated (190 active cases and 131 community deaths). North Kivu accounts for the majority of the volume with 852 alerts investigated (82.5%), while Ituri has 76.8% of its 531 alerts investigated, and South Kivu has only investigated 10 out of 55 (18.2%). Haut-Uélé has investigated all 22 of its alerts. Tshopo has not reported its alerts for July 31: its indicators remain unavailable.

3.2 Laboratory: tests and positivity

ŷ **Ituri** : 238 samples documented in the collection network, including 64 positive (55 live and 7 dead), — 53 new cases confirmed and 11 re-samples, representing an apparent positivity of 27.1%.

ŷ **North Kivu**: 127 samples received, of which 12 were positive (Beni 7, Butembo 5), representing a positivity rate of 9.4%; 44 analyses remain in progress.

ŷ **Haut-Uélé**: 17 samples received, of which 8 were analyzed: 4 positive results (Isiro 3 and Pawa 1) and 1 invalid result. Only one of the thirteen planned laboratories is operational, at the University Clinics of Uélé in Isiro; the facilities in Wamba and Boma Mangbetu are scheduled to open, and a 10 kW solar kit has been received from the WHO.

ŷ **Tshopo**: No new cases were reported for July 31; the provincial total remains at 7 positive results out of 47 tests. Non-compliant samples continue to be reported in the Lubunga and Opienge health zones, and samples continue to arrive at the laboratory without an investigation form.

ŷ **South Kivu**: 11 samples received, none positive. The Miti-Murhesa health zone remains the only affected area in the province.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri*	11,980	9,588	80.0%
North Kivu*	5,911	4,418	74.7%
Tshopo	ND	ND	ND
Haut-Uélé	562	375	66.7%
South Kivu	55	55	100.0%
Total	18,508	14,436	78.0%

* In Ituri and North Kivu, the follow-up rate was calculated only for health zones that reported. The health zones of Kambala, Damas, and Mandima (Ituri) and Rungu (Haut-Uélé) did not have usable data at the time of report consolidation. Tshopo did not report its contact tracing for July 31.

Contact tracing remains insufficient at the national level (78.0%), below the operational threshold of 95%, even though it has improved by 2.7 points compared to the previous day. Ituri has monitored 9,588 contacts out of 11,980 (80.0%), with the Kambala, Damas, and Mandima areas not reporting any data. North Kivu has seen 4,418 contacts out of 5,911 (74.7%), and Haut-Uélé 375 out of 562 (66.7%), with the Rungu area still lacking usable data. South Kivu, which is resuming contact tracing after several cycles of interruption, has seen all 55 of its contacts (100%). Tshopo has not reported its data for July 31.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 4 alerts notified

Four alerts were notified at provincial border crossing points, two of which were confirmed as live; none were referred to the CT/CTE. One body was intercepted and swabed (100%). Four Points of Control (PoCs) — Mud'zibala, Nyamusasi, Mandje, and Irumu Centre — did not transmit their data, with a completeness rate of 93.3% (56/60). Of the 102,561 travelers screened, 96.9% were screened, 96.5% washed their hands, and 99.5% received awareness training.

North Kivu — 2 alerts notified

Two alerts were notified: a live alert at the Matombo PoC, validated and referred to the CT/CTE (100%), and a body without Life intercepted at the Mukulia PoC. Fifteen of the sixteen PoE/PoCs reported, with the Makeke PoC not reporting (93.8% completeness). Of the 59,834 travelers screened, 97.4% were screened, washed their hands, and received awareness training.

Tshopo — not reported

No PoE/PoC slides are included in the July 31st deck: the province's indicators are marked ND in Table 5, with the exception of the proportion of activated PoCs (4 out of 19), confirmed by the challenges slide. The latest available data, published in the previous report, was from July 29th.

Haut-Uélé — 0 alerts notified

No alerts were notified to the province's activated PoE/PoC systems. The PoE/PoC pillar slides are empty in the July 31 deck—a fact confirmed after file conversion—so the number of travelers screened and the proportions of screening, handwashing, and awareness campaigns are marked ND in Table 5.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16)	100% (9/9) 21.1% (4/19)	ND	ND	85.6%*
Number of people who have switched to PoE/PoC	102,561	59,834	83,848	ND	ND	246 243
% of travelers screened	96.9%	97.4%	100%	ND	ND	98.1%*
% of travelers who washed their hands	96.5%	97.4%	100%	ND	ND	97.9%*
% of travelers made aware	99.5%	97.4%	100%	ND	ND	99.1%*
Number of alerts notified	4	2	0	ND	0	6
Number of validated live alerts	2	1	0	ND	0	3
Number of bodies recovered today	1	1	0	ND	0	2
Number of problematic contacts intercepted today	0	0	0	ND	0	0
Percentage of validated live alerts referred to the CT/CTE	0% (0/2)	100% (1/1)	0	ND	0	33.3% (1/3)
% of lifeless bodies swabbed	100% (1/1)	ND	0	ND	0	ND
Number of positive results of referred suspected cases	0	0	0	ND	0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator	Ituri	North Kivu	Haut-Uele	Tshopo	Together
Number of beds	821	141	68	20	1,050
Patients in bed (Day -1)	518	190	22	1	731
Total admissions (24 hours)	73	47	ND	1	121
Total admissions	844	368	47	2	1,261
Deceased	14	4	ND	0	18
No case	38	33	ND	0	71
Healed	13	2	ND	0	15
Escapes	1	1	ND	0	2
Transferred to the HGR	0	0	ND	0	0
Total outflows (24 h)	66	40	ND	0	106
Patients in isolation (end of day),	525	196	37	2	760
including confirmed cases	304	27	16	1	348
including suspects	221	169	21	1	412
Occupancy rate	63.9%	139.0%	54.4%	10.0%	72.4%

In total, 760 patients were in isolation, including 348 confirmed cases and 412 suspected cases, for an overall occupancy rate of 72.4% (760 patients for 1,050 beds), with 121 admissions and 106 discharges (15 recovered, 18 deceased, 71 non-cases, and 2 who escaped). Ituri accounts for the majority of the burden with 525 patients in isolation (304 confirmed, 221 suspected) out of 821 beds, representing 63.9% occupancy. In the North-

In Kivu, 196 patients are in isolation (27 confirmed, 169 suspected) for an occupancy rate of 139.0% (141 beds, capacity not yet reached). reported over five cycles); the daily movement does not close to the nearest unit, and the value retained is that of the detail by zone. Haut-Uélé has 37 patients in isolation (16 confirmed, 21 suspected) for 68 beds distributed across five CTEs (54.4%), with the Wamba CTE exceeding its capacity for confirmed cases (125%); its admissions and discharges for the day are noted as ND, as the care pillar did not report. Tshopo has 2 patients in isolation for 20 beds (10.0%). Despite a provincial occupancy rate of 63.9% in Ituri, six facilities remain above or at the limit of their capacity for confirmed cases: Nizi CTE (322%), Lita CTE (140%), ISTM CTE (107%), Bunia CTE (105%), CME CTE (100%), and Fataki CTE (100%); three CTs are saturated for suspects: Logo (125%), Kigonze (113%) and HGR Kilo (100%).

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	19 (76.0%)	5 (33.3%)
Confirmed cases are being monitored.	145 (71.1%)	8 (100.0%)
New suspected cases	23 (40.4%)	56 (57.1%)
Suspected cases are being monitored	120 (71.4%)	45 (68.2%)
Laboratory results announced (including positive ones)	50 (17)	48 (15)
Children separated — nursery / community	1/76	0/7
CTE support staff	354	34
Frontline Personnel (PPL)	15	40
Members of households and communities	93	65
EDS supported (SMSPS)	19	7

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE WORKERS (FWs) INFECTED WITH EVD IN ITURI

Health Zone	Confirmed cases	Cured. In hospital.		Death
Bunia	35	19	1	14
Rwampara	33	25	1	7
Mongbwalu	21	10	2	9
Nyankunde	14	11	1	2
Nia-Nia	20	1	13	4
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	4	2	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	3	0	3	0
Total — case fatality	141	72	25	41

29.1% PPL = healthcare workers and frontline providers.

In Ituri, 141 cases of PPL (Patients of Life-Throughput Disorder) are confirmed, including 41 deaths, representing a case fatality rate of 29.1%. The breakdown by health zone shows only 138 cases (72 recovered, 25 hospitalized, and 41 deaths): three cases remain unassigned and must be assigned by the provincial division.

of health. In North Kivu, 11 PPL are confirmed including 3 deaths; Haut-Uélé reports 3, or 5.3% of its confirmed cases, including one hospitalized at the CTE in Isiro.

7 Pillar-Based Response Activities

COORDINATION Holding of the coordination meeting for the response and monitoring of activities.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Continuation of the assessment of free healthcare, phase 1: the free healthcare unit is established (PTF, INSP, DPS, CSSE pillar, EUP Fass, CDR), hotspot zones and structures are prioritized, the free healthcare package is defined, and the partner mapping is complete. 163 health establishments have been pre-selected in 137 health areas across 36 zones, including 92 state-run, 48 faith-based, and 23 private; 43 of them already receive support from a partner. The costing establishes a monthly requirement of USD 2,209,095 to USD 3,976,371 (77 ESS PCAs for USD 1,842,329 and 86 ESS PMAs for USD 366,766). The official directive prohibiting any payments at the targeted healthcare facilities and the contracting with stakeholders remain unimplemented. Phase 2, "Health Security," includes triage at the entrance, a separate circuit for suspected cases, and rapid SOP/MEGS/PCI training.
LOGISTICS ÿ Ituri: Receipt of a used 20 KVA generator and a computer kit (computer, multitasking printer, overhead projector), donations from UNICEF to the Surveillance, IPC, and CREC pillars. Delivery of IPC and PECH kits to the health zones of Aungba, Kambala, Lolwa, Mambasa, Angumu, Fataki, Damas, and Rimba; IPC kits to the SGI depot and the Nia-Nia CTE; and PECH equipment to the Bunia CME CTE; Africa CDC provided a standard Starlink and five mini Starlink units. The fleet comprises 38 vehicles and 10 ambulances distributed among the pillars and nine health zones; 320 liters of fuel were provided to the Safety and Security pillar. Of the 26 ambulances identified, 23 are available and 3 are out of service (1 in Bambu, 1 with the FARDC, and 1 in the Rwampara health zone). 33 trips were made, 29 of which were successful, resulting in 25 patients being evacuated, with 4 refusals in Bunia. Work is underway at the Rwankole CTE (tiling, paving and electrical installation) and expansion tents are being erected at the Nia-Nia CTE. ÿ North Kivu: Delivery of IPC supplies to the Musienene Health Zone; receipt of medicines (UG-PDSS donation) and dignity kits and buckets (UNFPA donation); deployment of pillar teams to the health zones. Logistics indicators remain poor: 50 operational motorcycles out of 100 expected, 6 vehicles out of 7, 5	PCI / WASH ÿ Ituri: Training began for 50 healthcare providers and hygiene workers in the Nizi health zone with support from FHI 360; distribution of Category 3 IPC-WASH kits to three health facilities (CBCA Health Center, CNCA Health Center, and Bigo Health Center) with support from PPSSP/UNICEF, and 10 handwashing stations at the Governorate 2 market (FAEVU support). 25 rings opened around cases and 28 sites identified, all decontaminated within 48 hours, representing a 100% implementation rate; decontamination of 27 of the 69 households with cases within 48 hours (39.1%), mainly in Nizi (14), Bunia (5), Rwampara (3), Mongbwalu (2), and Mandima (1); 39 community health workers monitored and supported (Bunia 25, Nizi 7, Rwampara 7) out of a target of 394; 180 providers trained (Bunia 140, Bambu 27, Drodoro 13). The completeness of IPC reporting remains low, at 10 out of 29 health zones (34%). ÿ North Kivu: Continued training of 20 IPC/WASH focal points and 20 supervisors in the Kalunguta Health Zone with WHO support; decontamination of one household of 14 confirmed cases in Musienene and of 3 health facilities (Katwa 1, Musienene 2); scorecard assessment of a health facility in Beni, average score 51.6%; monitoring and support of 67 health facilities (Beni 38, Oicha 24, Katwa 3, Kyondo 2) and briefing of 240 service providers during formative supervision sessions. Nine staff exposure risk assessments were conducted, eight of which were in

Ambulances out of 30, no hearses out of 20, 2 out of 11 CTEs built to standards and no CTs out of 36; no area is free from shortages of IPC inputs and only 4 out of 11 areas are supplied with holistic care inputs. Tshopo: water supply to the PoC PK 23, packaging and delivery of an IPC kit to the Wanie-Rukula Health Zone, visit to the construction site of the large-capacity CTE with COUSP, IMA and MSF; the fleet remains limited to 4 liaison vehicles, 4 motorcycles and 4 ambulances for a need of 10 vehicles and 27 motorcycles, and Village Reach has delivered only 500 of the 1,000 liters of diesel promised.	High risk. Completeness only reaches 4 out of 11 zones. Tshopo: PCI assessment of the Konga Konga displaced persons site, score 19.4%, decontamination of the site and the FONAREV dispensary, handover of an PCI kit and mattresses to the Madula Health Center and briefing of service providers on water triage and chlorination.
COMMUNICATION (RCCE / CREC) ŷ Ituri: 2,717 people reached during 5 home visits, 2,550 during 12 educational talks, 5,325 during 13 mass awareness campaigns, 4,182 in churches, mosques, and markets, and 1,282 during 10 community dialogues; 12 people engaged following advocacy efforts. Mass awareness campaign of approximately 300 people at the Hoho market (Rwampara Health Zone) to demystify body bags and chlorinated water through practical demonstrations, with support from WHO, REMED, Africa CDC, UNICEF, and FHI 360. 104 community alerts reported (100 living, 4 deaths); 14 instances of resistance recorded, none overcome, all in the Bunia Health Zone; 35 feedback points identified, 5 of which were clarified (25%). Capacity building: 35 young leaders in Mambasa (IMA), 26 IT, ITA and ECZ members in Mongbwalu (Africa CDC), 30 RECO in Nizi (MEDAIR, FHI 360) and 42 Caritas agents in Bunia (CRS funding). Two radio programs produced, six spots broadcast, 36 community radio stations involved. ŷ North Kivu: Involvement of the mayor of Bungulu commune in raising awareness within his community about prevention measures and the safety of intervention teams. Briefing of 17 managers of private health facilities in Rwenzori commune (Beni Health Zone) on CREC, community-based surveillance, and standard precautions; capacity building for 35 writing club facilitators, AVEC members, and psychosocial workers in Vuhovi; prevention radio broadcast in Njiapanda (Biena Health Zone) addressing 76 listener concerns; community dialogue in Itebero with leaders and managers of mining sites along the forest and Kahuzi-Biega National Park corridor. 20,825 households visited and 33,785 people reached through door-to-door visits, 4,402 in public places, and 1,271 during dialogues and discussions; 1	HOLISTIC CARE (HCC) ŷ Ituri: 13 new recoveries; 525 patients in isolation (304 confirmed, 221 suspected), 73 new admissions and 66 discharges, including 14 deaths, 38 non-cases, and 1 escapee; 993 hot meals served (139 confirmed, 129 suspected, 690 contacts, and 35 people with HIV), 20 specific diets, and 8 infants under 6 months receiving infant formula. The cumulative number of recoveries has reached 611. A pillar meeting was held with partners, patients were transferred from ESS, CT, and the community to CTEs, results were announced, and the expansion and construction work on the CTEs continued; training of providers in GIH-mhGAP and monitoring of PECH data entry in DHIS2 at the Bunia General Hospital CTE. ŷ North Kivu: Cumulative total of 142 confirmed cases admitted to Ebola Treatment Centers (ETCs) with 48 deaths, representing an ETC mortality rate of 34%, and 13 lost to follow-up. 47 admissions today and 40 discharges (4 deaths, 33 non-cases, 2 recoveries, and 1 escapee), for a total of 196 patients in isolation at the end of the day (27 confirmed, 169 suspected). Hot meals served to 74 confirmed cases, 328 suspected cases, 100 frontline healthcare providers, and 294 caregivers, and 152 households supported through general food distribution; a separated infant is receiving breast milk substitute care in Kalunguta. Training of isolation providers at Beni General Referral Hospital and Bundji and Boikene Health Centers with the support of IMC, evaluation mission to Musienene and joint APATEMACO/PAM supervision in Butembo and Katwa. ŷ Haut-Uélé and Tshopo: 2 confirmed cases were admitted (Boma Mangbetu 1, Wamba 1) and 5 suspected cases (Wamba 3, Pawa 2), for a total of 37 patients hospitalized at the end of the day (16 confirmed, 21 suspected). Admissions and discharges for the day were not reported. The Wamba Ebola Treatment Center is operating at 125% capacity with confirmed cases, and the Boma Mangbetu Ebola Treatment Center is understaffed. Clinic Visits

199 people engaged and 210 alerts of suspected cases reported by community actors; all 34 targeted radio stations broadcast. ŷ Tshopo: Pillar meeting with situation analysis and deployment of teams by area (12 participants); advocacy with authorities and influencers in Kisangani for contact tracing of the 7th case (15 participants); focus group with managers of the Konga Konga IDP camp (27 participants); community dialogue and awareness-raising at the IDP camp, reaching 867 people. Support for training rapid response teams, follow-up to 7 community alerts in Mangobo, and continued dissemination of public service announcements. Haut-Uélé: continued mass awareness campaigns in churches, reaching 3,816 people through home visits in 1,272 households, 1,426 through mass awareness campaigns, and 63 through discussions; 3 of the 14 targeted radio stations broadcast; commencement of translation of IEC tools into Mayogo and Mangbetu.	University staff, from Libiki Hospital and the Tuluba Health Center, where a patient is being monitored. Tshopo: one new admission and no discharges, bringing the number of patients in isolation to 2; the confirmed case hospitalized at the Kisangani Hospital Center is stable on day three; experimental treatment continues for 3 high-risk contacts among frontline workers in the Madula health area (Wanie-Rukula Health Zone); the cumulative number of hospitalized confirmed cases stands at 4, including 2 deaths, 1 recovery, and 1 in intensive care. treatment course.
RESEARCH, INNOVATION & VACCINATION ŷ Ituri: Launch of Africa CDC's research activities by the Incident Manager, Professor Steve Ahuka. Pillar meeting and training of enumerators for the rapid qualitative survey with support from Africa CDC. Final session analyzing the challenges of the nine key pillars of the response and validation of the recommendations formulated, with support from WHO. Outlook: launch of the community survey after the enumerator briefing, organization of two conferences within the university community of Bunia, and continued receipt of partner protocols for analysis and certification. No vaccination activities were reported by the provinces today.	Dignified and secure burials (DSM) ŷ Ituri: 3,061 alerts were recorded for 2,496 EDS (Emergency Health Diagnosis) assessments carried out between May 20 and July 30. There were 72 new alerts and 24 postponements, with 54 EDS assessments completed (56%), 12 failures, and 31 postponed to the following day. 42 swabs were conducted, including 30 in communities and 12 in healthcare facilities. The alerts were distributed as follows: 47 community and household alerts, 14 in social and solidarity economy (SSE) settings, and 8 in treatment centers. Training began for two EDS teams in the Tchomia health zone, and training for the two teams in the Mangala health zone was completed. 18 EDS staff received psychosocial support. ŷ North Kivu: 33 death alerts received, 33 bodies swabbed and secured, and 31 DHS (Death Health Survey) carried out, without any failures; 16 swabbed and secured bodies remain pending. Deaths occurring in the community constitute the majority of alerts, primarily in Beni (14) and Katwa (12). 34 community deaths benefited from a DHS and 43 were swabbed.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ŷ Ituri: Briefing and awareness-raising began with focal points in the Fataki and Mandima health zones, and cascade briefings continued in the Ariwara health zone with WHO support. 62 people were briefed (56 men, 6 women), bringing the total to 2,659 people who have signed the commitment agreement in seven health zones, and 477 people were sensitized (223 men and boys, 254 women and girls), bringing the total to 15,806. Twelve Community-Based Community Management (CBCM) units were revitalized in Mongbwalu.	

8 Main challenges, impacts and actions required

Challenge No.		Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials	Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 139.0% in North Kivu (capacity of 141 beds: not reported for five cycles) and six structures beyond their capacity for confirmed cases in Ituri, including the Nizi CTE at 322% and the Lita CTE at 140%;	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Insufficient contact tracing performance: Ituri 80.0%, North Kivu 74.7%, Haut-Uélé 66.7% and Sud-Kivu 100%, with Tshopo not reporting; the health zones of Kambala, Damas and Mandima (Ituri) and Rungu (Haut-Uélé) without usable data	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs); strengthen supervision; consolidate the recovery observed in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (181/1170, i.e. 15%)	Risk of nosocomial infections (141 cases of PPL, including 41 deaths in Ituri; 11 PPL, including 3 deaths in North Kivu and 3 PPL confirmed at Haut-Uélé)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Rwampara, Fataki, Bambu-mine, Boga and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings of the resources	Limitation of the implementation of pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities





Public forum on perceptions of the fight against Ebola virus disease and community involvement in the response to this epidemic at the RUGHENDA/ AS RUGHENDA roundabout in the city of But

FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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